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Name: زينب علي عبد الظاهر عبد القادر National ID: 27712152100649	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Contrast-enhanced CT of chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين


 وزارة التعليم والبحث العلمي
 جامعة بغداد
 كلية التربية
 قسم التعليم المستمر
 رقم التسجيل: ٢٧٧ ١٢ ١٥ ٢١ ٠٠٦ ٤٩
 IS8415135


 وزارة التعليم والبحث العلمي
 جامعة بغداد
 كلية التربية
 قسم التعليم المستمر
 رقم التسجيل: ٢٧٧ ١٢ ١٥ ٢١ ٠٠٦ ٤٩
 المنطقة سارية حتى: ٢٠٢١/٠١/٠١

Aug 752

وزارة الصحة و السكان
مديرية الشؤون الصحية بمحافظة القاهرة
جوستاف روسي انترناشيونال - مصر (EPE)

فاتورة علاج رقم ١٧٧٨٢٨٦٠

تقر مستشفى جوستاف روسي انترناشيونال - مصر (EPE) بأن أصول مستندات الصرف ومزاداتها القانونية صحيحة وعلى مسئوليتنا. وتم حفظها بغرفة الحفظ لدينا وتحت تصرف الجهاز المركزي للمحاسبات واننا لم ولن نتقدم بتلك الفاتورة لأي جهة أخرى للصرف.

اسم المريض زينب على عبد الظاهر عبد القادر الرقم القومي ٢٧٧١٢١٥٢١٠٠٦٤٩
قرار وزاري رقم ٢٠٢٥٣٦١١٣٩٥٢ بتاريخ ٢٠٢٥-٠٧-٢٨ بمبلغ ٦٨٣٢ مدة القرار ١٨٠ يوم
تاريخ الفاتورة ٢٠٢٥-٠٨-١٧ التشخيص سرطان بالثدي (مبادرة)
الإجراء زولاديكس تحت الجلد لمدة 6 شهور ودوائي وابحات الخصم (%)

مدينة	بريد	كمية	وحدة	تاريخ الصرف	ملاحظات
٥٦٦٢.٨٦	Goserolin Acetate 10.8 mg	1	اخرى	17/08/2025	
٤.٤٨	Chlorpheniramine 5mg Injection	1	ampoule	17/08/2025	
٥.٦٠	Dexamethasone sod. PO4 8mg	1	AMP / 2 ML	17/08/2025	
٤٩.٩٦	NORMAL SALINE 0.9% 50ml	2	bottel	17/08/2025	
٥٧٢٢.٩٠	الاجملي				

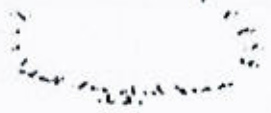
وتفضلوا بقبول فائق الاحترام،



رئيس حسابات المرضى
مصطفى ك

المراجع
طارق

الموظف المختص
ريتا كراي



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



A-Patient ID:
1- Name : زيات علي محمد الظاهر عبدالعزیز
2-ID number: 27712152100649

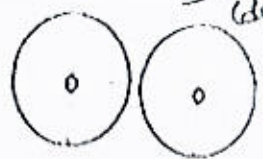
Sex:

MDT form
3- Medical number:
Female
Patient Age: 46 Y

83151607
Old Med:

2554-2023

MBC to L
Gda. no



Family history

marital status

Co morbidity

Phone

NRS Score

Menopausal status

01114547278

BMI

GROUP

A

REASON OF MDT

NEW

Date of MDT

2023-07-12 (3)

Breast Imaging :

Clinical data

Meta Workup:

DX:	ER:	PR:	Her 2 Neu :	SISH :	KI67 :
	+ve	+ve	-ve		
T	N		M		

Sonoinammography

PATIENT HISTORY

acase of denovo mets breast to bone 6/2020 , received 1st line AI with progression in breast
--> shifted to aromasine +palbo with SD dis.--> breast
CR--> bone
reffered for resuming treatment

MDT OUTCOME & PLAN

baseline cts & ttumor markers

continue aromasine +palbo +/- zoladex according to hormonal assay

MEDICAL ONCOLOGY	SIGNATURE	
SURGERY	02/07/23	RADIOLOGY
Psychiatric	02/07/23	PATHOLOGY
		Nutrition



Patient Name: Zainab Ali Abdel Zäher Abdel Kader	Date: 11/08/2024
Exam: C.T. CHEST WITH CONTRAST	Age: 46 Year(s)
Medical ID: 2654/2023	Pat. ID: 42287

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

Compared to previous study dated 25/9/2023; the current study revealed

I-Chest:

- Still noted bilateral axillary level I and II enlarged lymph nodes.
- Clear rest of both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Bilateral thyroid lobes hypodense nodules, for US.

II-Abdomen and pelvis:

- The liver is still seen mildly enlarged with diffuse homogenous low parenchymal CT texture. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.



Patient Name: Zainab Ali Abdel Zaher Abdel Kader	Date : 11/08/2024
Exam : C.T CHEST WITH CONTRAST	Age : 46 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

- Still seen calcified uterine fibroid and right adnexal lesion measures about 3.3x3.5 cm containing calcific foci? subserous fibroid.
- No abdominal or pelvic collections.

III-The scanned skeleton:

- Still seen multiple vertebral small lytic lesions yet no vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

Follow up status showing no gross time interval changes since study dated 25/9/2023.

Signature Dr. Walid Mohamed, MD

22/08/2024 06:10 ,

ALI, ZAYNEB Patient ID: 123022626 - DOB: 15-Dec-1977 Date: 03-May-2025
Patient Lastname: Ali
Patient Firstname: Zayneb
HIS Patient ID: 123022626
RIS Patient ID: 340505
Date of Birth: 15/12/1977

Procedure: 70030, Bone Scan
Medical Question:
Clinical Comment:

===== REPORT TEXT =====

99mTc MDP Bone Scan Report (3/5/2025)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of 99mTc MDP Revealed:

" No definite evidence of active osseous deposits could be seen over the surveyed skeleton.

" Degenerative changes are noted over the dorso-lumbar vertebrae.

" Arthritic changes are noted over both shoulder and both knee joints.

" Both kidneys are visualized

Conclusion:

" No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obliged
Mahmoud Akram
Mohamed Abdeltawab, MSc

===== REPORT TEXT ENDE =====



Histopathology report

Report Number: 353

Formaldehyde	التنوع	زيت على عدائنا	م الرصد
03/06/2020 12:46	الوقت	41 Y	سن
07/06/2020 13:05	وقت الاستلام		رؤى العينة
	تاريخ التقرير	صورة المرأة	المب التحليل
		صورة المرأة	نتيجة

Primary sample :

Right breast core biopsy

comment :

Gross examination :

4 cores of greyish white tissue (1.1)

Microscopic examination :

cores of breast tissue involved by invasive cords and trabeculae of small malignant cells with hyperchromatic nuclei that are arranged in non cohesive pattern

Referral Lab :

Comment :

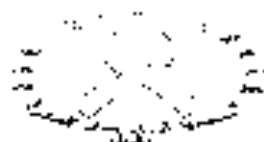
Diagnosis :

RIGHT BREAST IASIS B OPS / INVASIVE LOBULAR CARCINOMA, GRADE II

Pathologist

د/عبدالله بن دوي

Lab Director: د/فرزاد ماهر



Preclinical Program for Oncology Region
Ministry of Health and Planning

البرنامج السريري للأورام
وزارة الصحة والتخطيط



ERM036726062

MDT form

A-Patient ID:

1- Name : (أحمد عبد القادر عبد الله)

Sex:

3- Medical Number:

Female

83151607

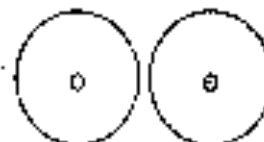
Old Med:

Date:

2554-2023

2-ID number:

Patient Age : 42 Y



Family history

First degree

Phone

Phone

011 14547 178

DM2

DM1

REASON OF MDT

REASON OF MDT

Current status

Current status

HRS Score

HRS Score

GROUP

GROUP

Date of MDT

Date of MDT

2024-12-02

Co morbidity

Co morbidity

Menopausal
status

Menopausal status

Initial STAGE

Initial STAGE

Clinical data

Breast Imaging :

partial radiological response of the rt breast malignant mass

BI-RADS 6

single left axillary indeterminate LN

Mets Workup :

bone scan: free

ER

ER

PR

HER2 Neu

ISH

Ki67

ER/PR/HER2

ER/PR/HER2

ER/PR/HER2

HER2 Neu/ISH

ISH/COMMENT

T

N

M

1 bone

Sonomammography

PATIENT HISTORY

case of de novo mets breast to bone 6/2020 , received 1st line AI with progression in breast

--> shifted to zoladex + aromasine + palbo received palbo for 3 months only

She was irregular on ttt and kept of Aromasin only with excellent response

bone scan: CR

CT CAP: CR

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

In view of CR,

keep on Zoladex + Aromasin + Zometa/3m

SIGNATURE

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

RADIOLOGY

RADIOLOGY

SURGERY

SURGERY

PATHOLOGY

PATHOLOGY

Psychiatric

Psychiatric

Nutrition

Nutrition

ALI, ZAYNEB Patient ID: 123022626 DOB: 15-Dec-1977 Date: 03-May-2025

Patient Lastname: Ali

Patient Firstname: Zayneb

HIS Patient ID: 123022626

RIS Patient ID: 3405115

Date of Birth: 15/12/1977

Procedure: 70030, Bone Scan

Medical Question:

Clinical Comment:

=====REPORT TEXT=====

99mTc MDP Bone Scan Report (3/5/2025)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I V Injection of 20mCi of

99mTc MDP Revealed:

* No definite evidence of active osseous deposits could be seen over the surveyed skeleton.

" Degenerative changes are noted over the dorso-lumbar vertebrae.

" Arthritic changes are noted over both shoulder and both knee joints.

" Both kidneys are visualized.

Conclusion:

" No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obligated

Mahmoud Akram

Mohamed Abdeltawab, MSc

=====REPORT TEXT ENDE=====



Histopathology report

Report Number: 355

Female	المرأة	زينة في عبداللطيف	بم المريض
المرأة	العمر	41 Y	السن
03/01/2020 12:46	تاريخ اصدار التقرير		تاريخ اخذ العينة
07/01/2020 13:08	تاريخ التقرير	صحة المرأة	طالب التحليل
		صحة المرأة	عناية

Primary sample :

Right breast core biopsy

Comment :

Gross examination :

4 cores of greyish white tissue

Microscopic examination :

Focal of breast tissue involved by invasive cords and trabeculae of atypical malignant cells with hyperchromatic nuclei that are arranged in non cohesive pattern

Referral Lab :

Comment :

Diagnosis :

RIGHT BREAST MASS B.C.C. INVASIVE LOBULAR CARCINOMA GRADE I

Pathologist:

د. فهد بن محمد

Lab Director:

د. فهد بن محمد

زسب على عبد الظاهر

^{99m}Tc MDP Bone Scan Report (29/5/2024)

• Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of ^{99m}Tc MDP **Revealed:**

- No definite evidence of active osseous deposits could be seen over the surveyed skeleton.
- Degenerative changes are noted over the dorso-lumbar vertebrae.
- Arthritic changes are noted over both shoulder and both knee joints.
- Both kidneys are visualized.

Conclusion:

- No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obligated

Mohamed Abdeltawab, MSc



Patient Name: Zainab Ali Abdel Zaher Abdel Kader	Date : 04/11/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 46 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

Combined bilateral breast mammography and complementary ultrasound

Indication:

Right breast pathologically proven malignant mass on NACT for assessment.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Complementary ultrasound revealed
- **Right breast:**
- No right breast residual suspicious solid mass lesions
- No pathological right axillary LNs
- Free Infra and supra-clavicular regions.
- **Left breast:**
- Single left axillary LN measured 2cm showing increased cortical thickness reaching 4mm
- No breast solid or cystic masses
- Normal skin thickness, and nipple-areola complex of the right breast.

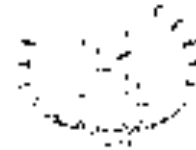
Opinion:

- Partial radiological response of the right breast malignant mass (BRADS 6)
- Single left axillary indeterminate LN for US follow up

Yours truly

Signature Dr. basma mohmaed

04/11/2024 3:24 PM



Patient Name: Zainab Ali Abdel Zaher Abdel Kader	Date : 11/08/2024
Exam : C.T: CHEST WITH CONTRAST	Age : 48 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

Compared to previous study dated 25/9/2023; the current study revealed

I-Chest:

- Still noted bilateral axillary level I and II enlarged lymph nodes.
- Clear rest of both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Bilateral thyroid lobes hypodense nodules, for US.

II-Abdomen and pelvis:

- The liver is still seen mildly enlarged with diffuse homogenous low parenchymal CT texture. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.



Patient Name: Zahab Ali Abdel Zaher Abdel Keder	Date : 11/08/2024
Exam : C.T. CHEST WITH CONTRAST	Age : 46 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

- Still seen calcified uterine fibroid and right adnexal lesion measures about 3.3x3.5 cm containing calcific foci? subserous fibroid.
- No abdominal or pelvic collections.

III-The scanned skeleton:

- Still seen multiple vertebral small lytic lesions yet no vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

Follow up status showing no gross time interval changes since study dated 25/9/2023.

Signature Dr. Walid Mohamed; MD

22/08/2024 06:10 ,

زسبب على عبد الظاهر

^{99m}Tc MDP Bone Scan Report (29/5/2024)

• Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of ^{99m}Tc MDP Revealed:

- No definite evidence of active osseous deposits could be seen over the surveyed skeleton.
- Degenerative changes are noted over the dorso-lumbar vertebrae.
- Arthritic changes are noted over both shoulder and both knee joints.
- Both kidneys are visualized.

Conclusion:

- No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obligated

Muhammed Abdeltawab, MSc



Patient Name: Zainab Ali Abdel-Zaher Abdel Kader	Date : 11/08/2024
Exam : C.T CHEST WITH CONTRAST	Age : 46 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

Compared to previous study dated 25/9/2023; the current study revealed

I-Chest:

- Still noted bilateral axillary level I and II enlarged lymph nodes.
- Clear rest of both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Bilateral thyroid lobes hypodense nodules, for US.

II-Abdomen and pelvis:

- The liver is still seen mildly enlarged with diffuse homogenous low parenchymal CT texture. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.



Patient Name: Zainab Ali Abdel Zaher Abdel Kader	Date : 17/08/2024
Exam: C.T CHEST WITH CONTRAST	Age : 46 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

- Still seen calcified uterine fibroid and right adnexal lesion measures about 3.3x3.5 cm containing calcific foci? subserous fibroid.
- No abdominal or pelvic collections.

III- The scanned skeleton:

- Still seen multiple vertebral small lytic lesions yet no vertebral structural collapse or extra-osseous soft-tissue component.

OPINION:

Follow up status showing no gross time interval changes since study dated 25/9/2023.

Signature: Dr. Walid Mohamed, MD

22/08/2024 06:10



Patient Name:	Zeinab Ali Abd El-Zaher abd elkader	Patient Gender:	Female
Patient Code:	Q35077	DATE:	1/15/2023 2:43:32 AM

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

Clinical history

- Female patient with history of right breast cancer.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post IV injection of 740 MBq of the tracer.

Findings

- No abnormal osseous tracer uptake is seen that would account for bone deposits.
- Arthritic changes of knees are noted.

N.B: focal tracer uptake is seen in the left side of the mandible likely attributed to dental problem for clinical correlation.

Opinion

- No convincing scintigraphic evidence of metastatic osseous lesions.

Dr. Merna Nasef

Dr. Eman Ashraf, MD



MULTISLICE TRIPHASIC C.T. SCAN OF THE ABDOMEN AND PELVIS:-

Multislice post contrast CT scan of the abdomen and pelvis with multiplanar reformatted images revealed:-

FINDINGS:-

- The liver is mildly enlarged showing diffuse fatty texture with no suspicious enhancing hepatic focal lesions detected.
- No intra or extrahepatic biliary dilatation.
- The portal vein is patent with no thrombosis.
- Normal CT appearance of the spleen, pancreas and both suprarenal glands.
- Both kidneys are functioning and normal.
- Normal filling of the urinary bladder with normal wall thickness and no calculi.
- Still noted bulky myomatous uterus with the largest one still seen subserous arising from its anterior fundal wall, currently measures 35.5x28mm (compared to 34x27mm previously), still showing dystrophic calcification within.
- Still noted enlarged heterogeneous both ovaries, with few foci of dystrophic calcification with the largest one currently seen on the right side measuring 31x31.5mm.
- No definite sizable colonic intraluminal soft tissue lesions detected.
- No ascites.
- Bone window still reveals widespread predominantly osteolytic lesions seen randomly distributed along both axial and appendicular skeletons, likely metastatic deposits.

IMPRESSION:-

- Mild fatty hepatomegaly.
- Still noted bulky myomatous uterus with bulky heterogeneous both ovaries, for further workup.
- Multiple widespread osteolytic lesions, mostly metastatic deposits.

PATIENT NAME: ZENEB ALI ABD AL ZAHAR ABD ALKADER	PATIENT ID #: 2020130609
GENDER: Female	ORDER #: 1087477
AGE: 43 y	DATE OF SERVICE: 01-Jun-2021 04:24 PM

Report:

99MTC-MDP BONE SCAN

Desired Information :- skeletal survey.

Technique:

Delayed Whole body imaging in anterior and posterior projection performed 2 minutes & 2 hours after I.V administration of 20 mCi 99mTc-MDP :

Findings:

Compared to previous bone scan dated 24/11/2020, the current study revealed rather stationary course of the previously seen wide spread osseous deposits which are seen involving the axial skeleton and proximal portions of long bones.

No newly seen lesions.

Both kidneys are seen.

Conclusion:-

Compared to 11/2020, the current study revealed rather stationary course of the wide spread metastatic osseous deposits.

Reported By,

Ass. Prof. Huda Fathy Elsayed



Patient Name: Zainab Ali Abdel Zaher Abdel Kader	Date : 09/12/2024
Exam : C.T CHEST	Age : 46 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

**NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

Compared to previous study dated 11/08/2024: the current study revealed

I-Chest:

- Still noted with stationary course of bilateral axillary level I and II enlarged lymph nodes.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Still seen prominent bilateral thyroid lobes with hypodense nodules.

II-Abdomen and pelvis:

- The liver is still seen mildly enlarged with diffuse homogenous low parenchymal CT texture. No definite focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Still seen calcified uterine fibroid and right adnexal lesion measures about 3.3x3.5 cm containing calcific foci? Sub-serous fibroid.
- No abdominal or pelvic collections.

III-The scanned skeleton:

- Still seen multi-level vertebral and left hemipelvic small lytic lesions yet no vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

Follow up status showing no gross time interval changes since study dated 11/08/2024.

Dr. Yasser Rashad, MSc

Dr. Mohamed Mohsen, M.D.



زینب علی حبیب الظاهر [25/9/2023]

وزارة الصحة والسكان
مقره للعركي الطبية المتخصصه
مستشفى (الأم دار السلام) - عمان

[illegible]

454.

15/12/2023

Premier. HBC to Bow

f 6/2023 IDC

En+ 1 PM+, 1 H2-u

1 H2-u 6/2023: 1st L AI → DP. 8/2023.

Zohar + Arang + Ibm < 3 ut

only 6 ut might be

NOT 2/12

Given 7 PCR Keep on Zohar, Arang 12/12

BS 1/2023 → 5/2024: f

CT. 8/2024: f



Doctor's Orders **أوامر الطبيب**

[illegible]

توقيع الطبيب	الأوامر	الوقت	التاريخ
3/9/2021	This pt. met. Br. Cener to have Referral from Knaif - Al - Hina to receive Amman + Ibance + 2 Amman Calcium One alpha قرى يوسف أحمد Lab. an. (3/9) AK Ibance 19/2 ميدالية الخارجى		

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

اسم المريض ثلاثي:

رقم الملف:



وزارة الصحة و السكان
المملكة العربية السعودية
مستشفى اورام دار السلام " هرمل "
إدارة الجودة

٤٥٦

Doctor's Orders الاوامر الطبية

التشخيص: نوع التقنية:

التاريخ	الوقت	الأوامر	توقيع الطبيب
		9/12/2024 - MDT 2/12/2024 Review CR Keep on Zoladex + Arimidex, Zoladex SM on 4/1/2025 partial Radiological Report of the Breast and pelvic lymphatic system BS 5/2/24 CR 3/12/24 venous flow for Dental ② No CT ③ Bone Scan	

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب



اسم المريض ثلاثي:

رقم الملف:

Doctor's Orders اوامر الطبيب

التشخيص: نوع التغذية:

التاريخ	الوقت	الأوامر	توقيع الطبيب
		Dentist Dear Dr, The patient has a tooth that needs to be extracted however she refused to do so and wishes to proceed with Zomeni Dr. Fatma Ali 15/1/2025	توقيع الطبيب

→ أتمسك على
15/1/25

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب



اسم المريض ثلاثي:

رقم الملف:

* HBC to Bone

Zaladex

اوامر الطبيب Doctor's Orders

Aromazin

Zometax / 3m

نوع التقنية:

التشخيص:

التاريخ	الوقت	الأوامر	توقيع الطبيب
		Bone Scan (R)	
		CT (R)	
		أقرب ما وجدته يومياً لمدة شهرين	
		كل أسبوعين	
		عند الحاجة	
		مكب	
		مستشفى التخصصية كسر بيه	
		أقرب ما وجدته يومياً	
		زوميتا (R)	
		ووجدت	
		Ca ⁺⁺ → 1.19	
		Sr → 0.8	
		Al → 0.7	
		Alc → 0.7	
		Zometax 4mg	
		(LBN) 2x 15	
		(2.5/1.5/2.5)	
		Bones can be free	
		waiting for C.T. result.	
		أقرب ما وجدته	
		كل أسبوعين	
		عند الحاجة	
		مكب	
		مستشفى التخصصية كسر بيه	
		أقرب ما وجدته يومياً	
		زوميتا (R)	
		ووجدت	

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

اسم المريض ثلاثي:
 رقم الملف:



وزارة الصحة و السكان
امانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام " هرمل"
إدارة الجودة

Doctor's Orders الأوامر الطبيب

الفحص: نوع التغذية:

التاريخ	الوقت	الأوامر	توقيع الطبيب
22/6/2025		Done Scan → CR (No active lesion) R/Axillary 1x1x30 R/Ovar Aden 1x1x90 R/Calcaneus 1x1x90 22/6/2025 [2x4.2ml of 2 units] أول 4 ديسا روغنيثا ٤ ورنيث زولاكس ٨/٨ عتاكس بنادول ٥ ٧x٥x١ سعيد C. C. A. IV سراج محمد	

Clinical pharmacy
Zameh Zaker
رئيس الصيدلاني
C. C. A. IV

جميع الأوامر الطبية لا بد أن تكون واضحة وموقعة من الطبيب

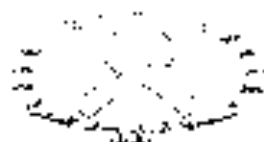


وزارة الصحة و السكان
إمالة المراكز الطبية المتخصصة
مستشفى أورام دار السلام " هرمل "
إدارة الجودة

أوامر الطبيب Doctor's Orders

اسم المريض ثلاثي:
عند المرض المعوي

رقم الملف :



Prehabilitation Program for Oncology Patients
Ministry of Health and Medical Education

البرنامج التمهيدى لمرضى الأورام
وزارة الصحة والتعليم الطبي



ERM036726062

MDT form

A-Patient ID:

1- Name : (أحمد بن عبد القادر عبد الله)

Sex:

3- Medical Number:

Female

83151607

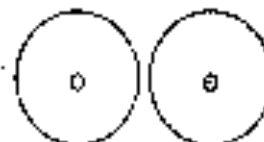
Old Med:

Date:

2554-2023

2-ID number:

Patient Age : 42 Y



Family history

First degree

Phone

Phone

05114547178

DM2

DM1

REASON OF MDT

REASON OF MDT

Current status

Current status

HRS Score

HRS Score

GROUP

GROUP

Date of MDT

Date of MDT

2024-12-02

Co morbidity

Co morbidity

Menopausal

status

Menopausal

Initial STAGE

Initial STAGE

Clinical data

Breast Imaging :

partial radiological response of the rt breast malignant mass

BI-RADS 6

single left axillary indeterminate LN

Mets Workup :

bone scan: free

ER

ER

PR

HER2 Neu

ISH

Ki67

ER/PR/HER2

ER/PR/HER2

ER/PR/HER2

HER2 Neu/ISH

ISH/COMMENT

T

N

M

1 bone

Sonomammography

PATIENT HISTORY

case of de novo mets breast to bone 6/2020 , received 1st line AI with progression in breast

--> shifted to zoladex + aromasine + palbo received palbo for 3 months only

She was irregular on ttt and kept of Aromasin only with excellent response

bone scan: CR

CT CAP: CR

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

In view of CR,

keep on Zoladex + Aromasin + Zometa/3m

SIGNATURE

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

RADIOLOGY

RADIOLOGY

SURGERY

SURGERY

PATHOLOGY

PATHOLOGY

Psychiatric

Psychiatric

Nutrition

Nutrition

ALI, ZAYNEB Patient ID: 123022626 DOB: 15-Dec-1977 Date: 03-May-2025

Patient Lastname: Ali

Patient Firstname: Zayneb

HIS Patient ID: 123022626

RIS Patient ID: 3405115

Date of Birth: 15/12/1977

Procedure: 70030, Bone Scan

Medical Question:

Clinical Comment:

=====REPORT TEXT=====

99mTc MDP Bone Scan Report (3/5/2025)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I V Injection of 20mCi of 99mTc MDP Revealed:

- * No definite evidence of active osseous deposits could be seen over the surveyed skeleton.
- " Degenerative changes are noted over the dorso-lumbar vertebrae.
- " Arthritic changes are noted over both shoulder and both knee joints.
- " Both kidneys are visualized.

Conclusion:

" No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obligated

Mahmoud Akram

Mohamed Abdeltawab, MSc

=====REPORT TEXT ENDE=====



Histopathology report

Report Number: 355

Female	المرأة	زينة في عبداللطيف	بم المريض
المرأة	العمر	41 Y	السن
03/01/2020 12:46	تاريخ اصدار التقرير		تاريخ اخذ العينة
07/01/2020 13:08	تاريخ التقرير	صحة المرأة	طالب التحليل
		صحة المرأة	عناية

Primary sample:

Right breast core biopsy

Comment:

Gross examination:

4 cores of greyish white tissue

Microscopic examination:

Focal of breast tissue involved by invasive cords and trabeculae of atypical malignant cells with hyperchromatic nuclei that are arranged in non cohesive pattern

Referral Lab:

Comment:

Diagnosis:

RIGHT BREAST MASS B.C.C. INVASIVE LOBULAR CARCINOMA GRADE I

Pathologist:

د. فهد بن محمد

Lab Director:

د. فهد بن محمد

زسب على عبد الظاهر

^{99m}Tc MDP Bone Scan Report (29/5/2024)

• Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of ^{99m}Tc MDP **Revealed:**

- No definite evidence of active osseous deposits could be seen over the surveyed skeleton.
- Degenerative changes are noted over the dorso-lumbar vertebrae.
- Arthritic changes are noted over both shoulder and both knee joints.
- Both kidneys are visualized.

Conclusion:

- No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obligated

Mohamed Abdeltawab, MSc



Patient Name: Zainab Ali Abdel Zaher Abdel Kader	Date : 04/11/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 46 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

Combined bilateral breast mammography and complementary ultrasound

Indication:

Right breast pathologically proven malignant mass on NACT for assessment.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Complementary ultrasound revealed
- **Right breast:**
- No right breast residual suspicious solid mass lesions
- No pathological right axillary LNs
- Free Infra and supra-clavicular regions.
- **Left breast:**
- Single left axillary LN measured 2cm showing increased cortical thickness reaching 4mm
- No breast solid or cystic masses
- Normal skin thickness, and nipple-areola complex of the right breast.

Opinion:

- Partial radiological response of the right breast malignant mass (BRADS 6)
- Single left axillary indeterminate LN for US follow up

Yours truly

Signature Dr. basma mohmaed

04/11/2024 3:24 PM



Patient Name: Zainab Ali Abdel Zaher Abdel Kader	Date : 11/08/2024
Exam : C.T: CHEST WITH CONTRAST	Age : 48 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

Compared to previous study dated 25/9/2023; the current study revealed

I-Chest:

- Still noted bilateral axillary level I and II enlarged lymph nodes.
- Clear rest of both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Bilateral thyroid lobes hypodense nodules, for US.

II-Abdomen and pelvis:

- The liver is still seen mildly enlarged with diffuse homogenous low parenchymal CT texture. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.



Patient Name: Zahab Ali Abdel Zaher Abdel Keder	Date : 11/08/2024
Exam : C.T CHEST WITH CONTRAST	Age : 46 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

- Still seen calcified uterine fibroid and right adnexal lesion measures about 3.3x3.5 cm containing calcific foci? subserous fibroid.
- No abdominal or pelvic collections.

III-The scanned skeleton:

- Still seen multiple vertebral small lytic lesions yet no vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

Follow up status showing no gross time interval changes since study dated 25/9/2023.

Signature Dr. Walid Mohamed; MD

22/08/2024 06:10 ,

زسبب على عبد الظاهر

^{99m}Tc MDP Bone Scan Report (29/5/2024)

• Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of ^{99m}Tc MDP Revealed:

- No definite evidence of active osseous deposits could be seen over the surveyed skeleton.
- Degenerative changes are noted over the dorso-lumbar vertebrae.
- Arthritic changes are noted over both shoulder and both knee joints.
- Both kidneys are visualized.

Conclusion:

- No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obligated

Muhammed Abdeltawab, MSc



Patient Name: Zainab Ali Abdel-Zaher Abdel Kader	Date : 11/08/2024
Exam : C.T CHEST WITH CONTRAST	Age : 46 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

Compared to previous study dated 25/9/2023; the current study revealed

I-Chest:

- Still noted bilateral axillary level I and II enlarged lymph nodes.
- Clear rest of both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Bilateral thyroid lobes hypodense nodules, for US.

II-Abdomen and pelvis:

- The liver is still seen mildly enlarged with diffuse homogenous low parenchymal CT texture. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.



Patient Name: Zainab Ali Abdel Zaher Abdel Kader	Date : 17/08/2024
Exam: C.T CHEST WITH CONTRAST	Age : 46 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

- Still seen calcified uterine fibroid and right adnexal lesion measures about 3.3x3.5 cm containing calcific foci? subserous fibroid.
- No abdominal or pelvic collections.

III- The scanned skeleton:

- Still seen multiple vertebral small lytic lesions yet no vertebral structural collapse or extra-osseous soft-tissue component.

OPINION:

Follow up status showing no gross time interval changes since study dated 25/9/2023.

Signature: Dr. Walid Mohamed, MD

22/08/2024 06:10



Patient Name:	Zeinab Ali Abd El-Zaher abd elkader	Patient Gender:	Female
Patient Code:	Q35077	DATE:	1/15/2023 2:43:32 AM

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

Clinical history

- Female patient with history of right breast cancer.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post IV injection of 740 MBq of the tracer.

Findings

- No abnormal osseous tracer uptake is seen that would account for bone deposits.
- Arthritic changes of knees are noted.

N.B: focal tracer uptake is seen in the left side of the mandible likely attributed to dental problem for clinical correlation.

Opinion

- No convincing scintigraphic evidence of metastatic osseous lesions.

Dr. Merna Nasef

Dr. Eman Ashraf, MD



MULTISLICE TRIPHASIC C.T. SCAN OF THE ABDOMEN AND PELVIS:-

Multislice post contrast CT scan of the abdomen and pelvis with multiplanar reformatted images revealed:-

FINDINGS:-

- The liver is mildly enlarged showing diffuse fatty texture with no suspicious enhancing hepatic focal lesions detected.
- No intra or extrahepatic biliary dilatation.
- The portal vein is patent with no thrombosis.
- Normal CT appearance of the spleen, pancreas and both suprarenal glands.
- Both kidneys are functioning and normal.
- Normal filling of the urinary bladder with normal wall thickness and no calculi.
- Still noted bulky myomatous uterus with the largest one still seen subserous arising from its anterior fundal wall, currently measures 35.5x28mm (compared to 34x27mm previously), still showing dystrophic calcification within.
- Still noted enlarged heterogeneous both ovaries, with few foci of dystrophic calcification with the largest one currently seen on the right side measuring 31x31.5mm.
- No definite sizable colonic intraluminal soft tissue lesions detected.
- No ascites.
- Bone window still reveals widespread predominantly osteolytic lesions seen randomly distributed along both axial and appendicular skeletons, likely metastatic deposits.

IMPRESSION:-

- Mild fatty hepatomegaly.
- Still noted bulky myomatous uterus with bulky heterogeneous both ovaries, for further workup.
- Multiple widespread osteolytic lesions, mostly metastatic deposits.

PATIENT NAME: ZENEB ALI ABD AL ZAHAR ABD ALKADER	PATIENT ID #: 2020130609
GENDER: Female	ORDER #: 1087477
AGE: 43 y	DATE OF SERVICE: 01-Jun-2021 04:24 PM

Report:

99MTC-MDP BONE SCAN

Desired Information :- skeletal survey.

Technique:

Delayed Whole body imaging in anterior and posterior projection performed 2 minutes & 2 hours after I.V administration of 20 mCi 99mTc-MDP :

Findings:

Compared to previous bone scan dated 24/11/2020, the current study revealed rather stationary course of the previously seen wide spread osseous deposits which are seen involving the axial skeleton and proximal portions of long bones.

No newly seen lesions.

Both kidneys are seen.

Conclusion:-

Compared to 11/2020, the current study revealed rather stationary course of the wide spread metastatic osseous deposits.

Reported By,

Ass. Prof. Huda Fathy Elsayed



Patient Name: Zainab Ali Abdel Zaher Abdel Kader	Date : 09/12/2024
Exam : C.T CHEST	Age : 46 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

**NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

Compared to previous study dated 11/08/2024: the current study revealed

I-Chest:

- Still noted with stationary course of bilateral axillary level I and II enlarged lymph nodes.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Still seen prominent bilateral thyroid lobes with hypodense nodules.

II-Abdomen and pelvis:

- The liver is still seen mildly enlarged with diffuse homogenous low parenchymal CT texture. No definite focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Still seen calcified uterine fibroid and right adnexal lesion measures about 3.3x3.5 cm containing calcific foci? Sub-serous fibroid.
- No abdominal or pelvic collections.

III-The scanned skeleton:

- Still seen multi-level vertebral and left hemipelvic small lytic lesions yet no vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

Follow up status showing no gross time interval changes since study dated 11/08/2024.

Dr. Yasser Rashad, MSc

Dr. Mohamed Mohsen, M.D.

454.

15/12/2023

Premier. HBC to Bow

f 6/2023 IDC

En+ 1 PM+, 1 H2-u

1 H2-u 6/2023: 1st L AI → DP. 8/2023.

Zohar + Arang + Ibm < 3 ut

only 6 ut might be

NOT 2/12

Given 7 PCR Keep on Zohar, Arang 12/12

BS 1/2023 → 5/2024: f

CT. 8/2024: f

اسم المريض ثلاثي: زينب عبد الله عبد الحار
رقم الملف: ٢٠٢٣ / ٢٥٥٤



وزارة الصحة و السكان
امانة المراكز الطبية المتخصصة
مستشفى أودام دار السلام " هرمل "
إدارة الجودة

Doctor's Orders الوامر الطبيب

التشخيص: نوع التغذية:

التاريخ	الوقت	الأوامر	توقيع الطبيب
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3/9/2023		Thin pt. met. Br. Cancer 1st line Referral for Kwik-Al-Bin to receive Aramox + Ibupro + Zoladex Aramox Calcimete On alpha	
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Ar. 3/9/23

Ar. 3/9/23

Ar. 3/9/23
Ibupro
Ar. 3/9/23

Ar. 3/9/23

Ar. 3/9/23
Ibupro
Ar. 3/9/23

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

اسم المريض ثلاثي:
رقم الملف:



وزارة الصحة و السكان
امانة المراكز الطبية المتخصصة
مستشفى اورام دار السلام " هرمل "
إدارة الجودة

Doctor's Orders أوامر الطبيب

التشخيص: نوع النخبة:

التاريخ	الوقت	الأوامر	توقيع الطبيب
		ABC 1- إعطاء 100 مل من فيتامين ك 1 2- إعطاء 100 مل من فيتامين ك 2 3- إعطاء 100 مل من فيتامين ك 3 4- إعطاء 100 مل من فيتامين ك 4 5- إعطاء 100 مل من فيتامين ك 5 6- إعطاء 100 مل من فيتامين ك 6 7- إعطاء 100 مل من فيتامين ك 7 8- إعطاء 100 مل من فيتامين ك 8 9- إعطاء 100 مل من فيتامين ك 9 10- إعطاء 100 مل من فيتامين ك 10	

د. أحمد
٢٠٢٢

Dr. Ahmad
[no. 125]

Lab ok for
Ibrance 125
C. 125

د. أحمد
٢٠٢٢

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

اسم المريض ثلاثي:

رقم الملف:



وزارة الصحة و السكان
المملكة العربية السعودية
مستشفى اورام دار السلام " هرمل "
إدارة الجودة

٤٥٦

Doctor's Orders الاوامر الطبية

التشخيص: نوع التقنية:

التاريخ	الوقت	الأوامر	توقيع الطبيب
		9/12/2024 - MDT 2/12/2024 Review CR Keep on Zoladex + Arimidex, Zoladex 3x SM on 4/11/2024 partial Radiological Resp of Rx BSC, no pt. in high efficacy, no BSC 5/2024 CR 3/2024 ven. ly. Res for O.Dental ② No CR ③ Bone Scan	

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

اسم المريض ثلاثي:
 رقم الملف:



وزارة الصحة و المكان
أمانة المراكز الطبية المتخصصة
مستشفى أوران دار السلام " هرميل"
إدارة الجودة

اوامر الطبيب Doctor's Orders

التشخيص: نوع التغذية:

التاريخ	الوقت	الأوامر	توقيع الطبيب
		13/1/13 CA I: 1.15 14/1/13 15/1/13 16/1/13 17/1/13 18/1/13 19/1/13 20/1/13 21/1/13 22/1/13 23/1/13 24/1/13 25/1/13 26/1/13 27/1/13 28/1/13 29/1/13 30/1/13 31/1/13 1/2/13 2/2/13 3/2/13 4/2/13 5/2/13 6/2/13 7/2/13 8/2/13 9/2/13 10/2/13 11/2/13 12/2/13 13/2/13 14/2/13 15/2/13 16/2/13 17/2/13 18/2/13 19/2/13 20/2/13 21/2/13 22/2/13 23/2/13 24/2/13 25/2/13 26/2/13 27/2/13 28/2/13 29/2/13 30/2/13 31/2/13 1/3/13 2/3/13 3/3/13 4/3/13 5/3/13 6/3/13 7/3/13 8/3/13 9/3/13 10/3/13 11/3/13 12/3/13 13/3/13 14/3/13 15/3/13 16/3/13 17/3/13 18/3/13 19/3/13 20/3/13 21/3/13 22/3/13 23/3/13 24/3/13 25/3/13 26/3/13 27/3/13 28/3/13 29/3/13 30/3/13 31/3/13 1/4/13 2/4/13 3/4/13 4/4/13 5/4/13 6/4/13 7/4/13 8/4/13 9/4/13 10/4/13 11/4/13 12/4/13 13/4/13 14/4/13 15/4/13 16/4/13 17/4/13 18/4/13 19/4/13 20/4/13 21/4/13 22/4/13 23/4/13 24/4/13 25/4/13 26/4/13 27/4/13 28/4/13 29/4/13 30/4/13 31/4/13 1/5/13 2/5/13 3/5/13 4/5/13 5/5/13 6/5/13 7/5/13 8/5/13 9/5/13 10/5/13 11/5/13 12/5/13 13/5/13 14/5/13 15/5/13 16/5/13 17/5/13 18/5/13 19/5/13 20/5/13 21/5/13 22/5/13 23/5/13 24/5/13 25/5/13 26/5/13 27/5/13 28/5/13 29/5/13 30/5/13 31/5/13 1/6/13 2/6/13 3/6/13 4/6/13 5/6/13 6/6/13 7/6/13 8/6/13 9/6/13 10/6/13 11/6/13 12/6/13 13/6/13 14/6/13 15/6/13 16/6/13 17/6/13 18/6/13 19/6/13 20/6/13 21/6/13 22/6/13 23/6/13 24/6/13 25/6/13 26/6/13 27/6/13 28/6/13 29/6/13 30/6/13 31/6/13 1/7/13 2/7/13 3/7/13 4/7/13 5/7/13 6/7/13 7/7/13 8/7/13 9/7/13 10/7/13 11/7/13 12/7/13 13/7/13 14/7/13 15/7/13 16/7/13 17/7/13 18/7/13 19/7/13 20/7/13 21/7/13 22/7/13 23/7/13 24/7/13 25/7/13 26/7/13 27/7/13 28/7/13 29/7/13 30/7/13 31/7/13 1/8/13 2/8/13 3/8/13 4/8/13 5/8/13 6/8/13 7/8/13 8/8/13 9/8/13 10/8/13 11/8/13 12/8/13 13/8/13 14/8/13 15/8/13 16/8/13 17/8/13 18/8/13 19/8/13 20/8/13 21/8/13 22/8/13 23/8/13 24/8/13 25/8/13 26/8/13 27/8/13 28/8/13 29/8/13 30/8/13 31/8/13 1/9/13 2/9/13 3/9/13 4/9/13 5/9/13 6/9/13 7/9/13 8/9/13 9/9/13 10/9/13 11/9/13 12/9/13 13/9/13 14/9/13 15/9/13 16/9/13 17/9/13 18/9/13 19/9/13 20/9/13 21/9/13 22/9/13 23/9/13 24/9/13 25/9/13 26/9/13 27/9/13 28/9/13 29/9/13 30/9/13 31/9/13 1/10/13 2/10/13 3/10/13 4/10/13 5/10/13 6/10/13 7/10/13 8/10/13 9/10/13 10/10/13 11/10/13 12/10/13 13/10/13 14/10/13 15/10/13 16/10/13 17/10/13 18/10/13 19/10/13 20/10/13 21/10/13 22/10/13 23/10/13 24/10/13 25/10/13 26/10/13 27/10/13 28/10/13 29/10/13 30/10/13 31/10/13 1/11/13 2/11/13 3/11/13 4/11/13 5/11/13 6/11/13 7/11/13 8/11/13 9/11/13 10/11/13 11/11/13 12/11/13 13/11/13 14/11/13 15/11/13 16/11/13 17/11/13 18/11/13 19/11/13 20/11/13 21/11/13 22/11/13 23/11/13 24/11/13 25/11/13 26/11/13 27/11/13 28/11/13 29/11/13 30/11/13 31/11/13 1/12/13 2/12/13 3/12/13 4/12/13 5/12/13 6/12/13 7/12/13 8/12/13 9/12/13 10/12/13 11/12/13 12/12/13 13/12/13 14/12/13 15/12/13 16/12/13 17/12/13 18/12/13 19/12/13 20/12/13 21/12/13 22/12/13 23/12/13 24/12/13 25/12/13 26/12/13 27/12/13 28/12/13 29/12/13 30/12/13 31/12/13	

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب



اسم المريض ثلاثي:

رقم الملف:

Doctor's Orders اوامر الطبيب

التشخيص: نوع التغذية:

التاريخ	الوقت	الأوامر	توقيع الطبيب
		Dentist	
		Dear Dr,	
		The patient has a tooth that	
		needs to be extracted however she	
		refused to do so and wishes to	
		proceed with Zomeni	
		Dr. Fatima Ali	
		15/1/2025	

→ تم تسجيل على
15/1/25

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

اسم المريض ثلاثي:
 رقم الملف:



وزارة الصحة و السكان
امانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام " هرمل "
إدارة الجودة

Doctor's Orders الأوامر الطبيب

الفحص: نوع التغذية:

التاريخ	الوقت	الأوامر	توقيع الطبيب
22/6/2025		Done Scan → CR (No active lesion) R / Augmentin 1x1x30 R / Omeprazole 1x1x90 R / Calceos 1x1x90 22/6/2025 [Zoled 4 Zoled after 2 weeks] أول 4 ديسا روغيتا ٤ وردي زولاكس ٨/٨ عتاكس بنادول ٥ ٧x٥x١ سعيد C. C. A. IV ساجد مختار	

Clinical pharmacy
Zoled Zoled
رصيد
C. C. A. IV

جميع الأوامر الطبية لا بد أن تكون واضحة وموقعة من الطبيب

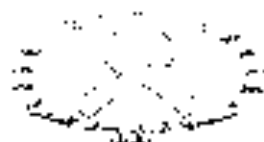


وزارة الصحة و السكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام " هرمل "
إدارة الجودة

أوامر الطبيب Doctor's Orders

اسم المريض ثلاثي:
عند المرض المعوي

رقم الملف :



Preclinical Program for Oncology Region
Ministry of Health and Planning

البرنامج السريري للأورام
وزارة الصحة والتخطيط



ERM036726062

MDT form

A-Patient ID:

1- Name : (أحمد بن عبد القادر عبد الله)

Sex:

3- Medical Number:

Female

83151607

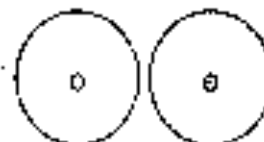
Old Med:

Date:

2554-2023

2-ID number:

Patient Age : 42 T



Family history

First degree

Phone

Phone

011 14547178

DM2

DM1

REASON OF MDT

REASON OF MDT

Current status

Current status

HRS Score

HRS Score

GROUP

GROUP

Date of MDT

Date of MDT

2024-12-02

Co morbidity

Co morbidity

Menopausal

status

Menopausal

Initial STAGE

Initial STAGE

Clinical data

Breast Imaging :

partial radiological response of the rt breast malignant mass

BI-RADS 6

single left axillary indeterminate LN

Mets Workup :

bone scan:free

ER

ER

PR

HER2 Neu

ISH

Ki67

ER/PR/HER2

ER/PR/HER2

ER/PR/HER2

HER2 Neu/ISH

ISH/COMMENT

T

N

M

1 bone

Sonomammography

PATIENT HISTORY

case of de novo mets breast to bone 6/2020 , received 1st line AI with progression in breast

--> shifted to zoladex + aromasine + palbo received palbo for 3 months only

She was irregular on ttt and kept of Aromasin only with excellent response

bone scan: CR

CT CAP: CR

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

In view of CR,

keep on Zoladex + Aromasin + Zometa/3m

SIGNATURE

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

RADIOLOGY

RADIOLOGY

SURGERY

SURGERY

PATHOLOGY

PATHOLOGY

Psychiatric

Psychiatric

Nutrition

Nutrition

ALI, ZAYNEB Patient ID: 123022626 DOB: 15-Dec-1977 Date: 03-May-2025

Patient Lastname: Ali

Patient Firstname: Zayneb

HIS Patient ID: 123022626

RIS Patient ID: 3405115

Date of Birth: 15/12/1977

Procedure: 70030, Bone Scan

Medical Question:

Clinical Comment:

=====REPORT TEXT=====

99mTc MDP Bone Scan Report (3/5/2025)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I V Injection of 20mCi of 99mTc MDP Revealed:

- * No definite evidence of active osseous deposits could be seen over the surveyed skeleton.
- " Degenerative changes are noted over the dorso-lumbar vertebrae.
- " Arthritic changes are noted over both shoulder and both knee joints.
- " Both kidneys are visualized.

Conclusion:

" No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obligated

Mahmoud Akram

Mohamed Abdeltawab, MSc

=====REPORT TEXT ENDE=====



Histopathology report

Report Number: 355

Female	المرأة	زينة في عبداللطيف	نام المريض
المرأة	العمر	41 Y	السن
03/01/2020 12:46	تاريخ اصدار التقرير		تاريخ اخذ العينة
07/01/2020 13:08	تاريخ التقرير	صحة المرأة	طالب التحليل
		صحة المرأة	عناية

Primary sample :

Right breast core biopsy

Comment :

Gross examination :

4 cores of greyish white tissue (1.1)

Microscopic examination :

Focal of breast tissue involved by invasive cords and trabeculae of atypical malignant cells with hyperchromatic nuclei that are arranged in non cohesive pattern

Referal Lab :

Comment :

Diagnosis :

RIGHT BREAST MASS B.C.C. INVASIVE LOBULAR CARCINOMA GRADE I

Pathologist:

د. أيمن محمد

Lab Director:

د. فهد محمد

زسب على عبد الظاهر

^{99m}Tc MDP Bone Scan Report (29/5/2024)

• Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of ^{99m}Tc MDP **Revealed:**

- No definite evidence of active osseous deposits could be seen over the surveyed skeleton.
- Degenerative changes are noted over the dorso-lumbar vertebrae.
- Arthritic changes are noted over both shoulder and both knee joints.
- Both kidneys are visualized.

Conclusion:

- No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obligated

Mohamed Abdeltawab, MSc



Patient Name: Zainab Ali Abdel Zaher Abdel Kader	Date : 04/11/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 46 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

Combined bilateral breast mammography and complementary ultrasound

Indication:

Right breast pathologically proven malignant mass on NACT for assessment.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Complementary ultrasound revealed
- **Right breast:**
- No right breast residual suspicious solid mass lesions
- No pathological right axillary LNs
- Free Infra and supra-clavicular regions.
- **Left breast:**
- Single left axillary LN measured 2cm showing increased cortical thickness reaching 4mm
- No breast solid or cystic masses
- Normal skin thickness, and nipple-areola complex of the right breast.

Opinion:

- Partial radiological response of the right breast malignant mass (BRADS 6)
- Single left axillary indeterminate LN for US follow up

Yours truly

Signature Dr. basma mohmaed

04/11/2024 3:24 PM



Patient Name: Zainab Ali Abdel Zaher Abdel Kader	Date : 11/08/2024
Exam : C.T: CHEST WITH CONTRAST	Age : 48 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

Compared to previous study dated 25/9/2023; the current study revealed

I-Chest:

- Still noted bilateral axillary level I and II enlarged lymph nodes.
- Clear rest of both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Bilateral thyroid lobes hypodense nodules, for US.

II-Abdomen and pelvis:

- The liver is still seen mildly enlarged with diffuse homogenous low parenchymal CT texture. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.



Patient Name: Zahab Ali Abdel Zaher Abdel Keder	Date : 11/08/2024
Exam : C.T. CHEST WITH CONTRAST	Age : 46 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

- Still seen calcified uterine fibroid and right adnexal lesion measures about 3.3x3.5 cm containing calcific foci? subserous fibroid.
- No abdominal or pelvic collections.

III-The scanned skeleton:

- Still seen multiple vertebral small lytic lesions yet no vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

Follow up status showing no gross time interval changes since study dated 25/9/2023.

Signature Dr. Walid Mohamed; MD

22/08/2024 06:10 ,

زسبب على عبد الظاهر

^{99m}Tc MDP Bone Scan Report (29/5/2024)

• Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of ^{99m}Tc MDP Revealed:

- No definite evidence of active osseous deposits could be seen over the surveyed skeleton.
- Degenerative changes are noted over the dorso-lumbar vertebrae.
- Arthritic changes are noted over both shoulder and both knee joints.
- Both kidneys are visualized.

Conclusion:

- No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obligated

Muhammed Abdeltawab, MSc



Patient Name: Zainab Ali Abdel-Zaher Abdel Kader	Date : 11/08/2024
Exam : C.T CHEST WITH CONTRAST	Age : 46 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

Compared to previous study dated 25/9/2023; the current study revealed

I-Chest:

- Still noted bilateral axillary level I and II enlarged lymph nodes.
- Clear rest of both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Bilateral thyroid lobes hypodense nodules, for US.

II-Abdomen and pelvis:

- The liver is still seen mildly enlarged with diffuse homogenous low parenchymal CT texture. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.



Patient Name: Zainab Ali Abdel Zaher Abdel Kader	Date : 17/08/2024
Exam: C.T CHEST WITH CONTRAST	Age : 46 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

- Still seen calcified uterine fibroid and right adnexal lesion measures about 3.3x3.5 cm containing calcific foci? subserous fibroid.
- No abdominal or pelvic collections.

III- The scanned skeleton:

- Still seen multiple vertebral small lytic lesions yet no vertebral structural collapse or extra-osseous soft-tissue component.

OPINION:

Follow up status showing no gross time interval changes since study dated 25/9/2023.

Signature: Dr. Walid Mohamed, MD

22/08/2024 06:10



Patient Name:	Zeinab Ali Abd El-Zaher abd elkader	Patient Gender:	Female
Patient Code:	Q35077	DATE:	1/15/2023 2:43:32 AM

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

Clinical history

- Female patient with history of right breast cancer.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post IV injection of 740 MBq of the tracer.

Findings

- No abnormal osseous tracer uptake is seen that would account for bone deposits.
- Arthritic changes of knees are noted.

N.B: focal tracer uptake is seen in the left side of the mandible likely attributed to dental problem for clinical correlation.

Opinion

- No convincing scintigraphic evidence of metastatic osseous lesions.

Dr. Merna Nasef

Dr. Eman Ashraf, MD



MULTISLICE TRIPHASIC C.T. SCAN OF THE ABDOMEN AND PELVIS:-

Multislice post contrast CT scan of the abdomen and pelvis with multiplanar reformatted images revealed:-

FINDINGS:-

- The liver is mildly enlarged showing diffuse fatty texture with no suspicious enhancing hepatic focal lesions detected.
- No intra or extrahepatic biliary dilatation.
- The portal vein is patent with no thrombosis.
- Normal CT appearance of the spleen, pancreas and both suprarenal glands.
- Both kidneys are functioning and normal.
- Normal filling of the urinary bladder with normal wall thickness and no calculi.
- Still noted bulky myomatous uterus with the largest one still seen subserous arising from its anterior fundal wall, currently measures 35.5x28mm (compared to 34x27mm previously), still showing dystrophic calcification within.
- Still noted enlarged heterogeneous both ovaries, with few foci of dystrophic calcification with the largest one currently seen on the right side measuring 31x31.5mm.
- No definite sizable colonic intraluminal soft tissue lesions detected.
- No ascites.
- Bone window still reveals widespread predominantly osteolytic lesions seen randomly distributed along both axial and appendicular skeletons, likely metastatic deposits.

IMPRESSION:-

- Mild fatty hepatomegaly.
- Still noted bulky myomatous uterus with bulky heterogeneous both ovaries, for further workup.
- Multiple widespread osteolytic lesions, mostly metastatic deposits.

PATIENT NAME: ZENEB ALI ABD AL ZAHAR ABD ALKADER	PATIENT ID #: 2020130609
GENDER: Female	ORDER #: 1087477
AGE: 43 y	DATE OF SERVICE: 01-Jun-2021 04:24 PM

Report:

99MTC-MDP BONE SCAN

Desired Information :- skeletal survey.

Technique:

Delayed Whole body imaging in anterior and posterior projection performed 2 minutes & 2 hours after I.V administration of 20 mCi 99mTc-MDP :

Findings:

Compared to previous bone scan dated 24/11/2020, the current study revealed rather stationary course of the previously seen wide spread osseous deposits which are seen involving the axial skeleton and proximal portions of long bones.

No newly seen lesions.

Both kidneys are seen.

Conclusion:-

Compared to 11/2020, the current study revealed rather stationary course of the wide spread metastatic osseous deposits.

Reported By,

Ass. Prof. Huda Fathy Elsayed



Patient Name: Zainab Ali Abdel Zaher Abdel Kader	Date : 09/12/2024
Exam : C.T CHEST	Age : 46 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

**NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

Compared to previous study dated 11/08/2024: the current study revealed

I-Chest:

- Still noted with stationary course of bilateral axillary level I and II enlarged lymph nodes.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Still seen prominent bilateral thyroid lobes with hypodense nodules.

II-Abdomen and pelvis:

- The liver is still seen mildly enlarged with diffuse homogenous low parenchymal CT texture. No definite focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Still seen calcified uterine fibroid and right adnexal lesion measures about 3.3x3.5 cm containing calcific foci? Sub-serous fibroid.
- No abdominal or pelvic collections.

III-The scanned skeleton:

- Still seen multi-level vertebral and left hemipelvic small lytic lesions yet no vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

Follow up status showing no gross time interval changes since study dated 11/08/2024.

Dr. Yasser Rashad, MSc

Dr. Mohamed Mohsen, M.D.

454.

15/12/2023

Premier. HBC to Bow

f 6/2023 IDC

En+ 1 PM+, 1 H2-u

1 H2-u 6/2023: 1st L AI → DP. 8/2023.

Zohar + Arang + Ibm < 3 ut

only 6 ut might be

NOT 2/12

Given 7 PCR Keep on Zohar, Arang 12/12

BS 1/2023 → 5/2024: f

CT. 8/2024: f

اسم المريض ثلاثي: زينب عبد الله عبد الحار
رقم الملف: ٢٠٢٣ / ٢٥٥٤



وزارة الصحة و السكان
امانة المراكز الطبية المتخصصة
مستشفى أودام دار السلام " هرمل "
إدارة الجودة

Doctor's Orders الأوامر الطبيب

التشخيص: نوع التغذية:

التاريخ	الوقت	الأوامر	توقيع الطبيب
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3/9/2023		Thin pt. met. Br. Cancer 1st line Referral for Kwik-Al-Bin to receive Aramase + Ibance + Zoladex Aramase Calcimete Onc alpha	
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أمر الطبيب

أمر الطبيب

Armsen (3/9) AK
Ibance
Calc 19/12

صيدلية الخارج

أمر الطبيب

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب



اسم المريض ثلاثي: رئيسة علي عبد رزاق م
رقم الملف:

Doctor's Orders أوامر الطبيب

التشخيص: نوع التغذية:

التاريخ	الوقت	الأوامر	توقيع الطبيب
		CT CAP: How a low S.O for long mto	
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صيدلية الخارجى

اسم المريض ثلاثي:

رقم الملف:



وزارة الصحة والسكان
المملكة العراقية الطبية المتخصصة
مستشفى اورام دار السلام " هرمل "
إدارة الجودة

٤٥٦

Doctor's Orders الاوامر الطبية

التشخيص: نوع التقنية:

التاريخ	الوقت	الأوامر	توقيع الطبيب
		9/12/2024 - MDT 2/12/2024 Review CR Keep on Zoladex + Arimidex, Zoladex SM on 4/11/2024 partial Radiological Resp of Rx BSC, no pt. in high efficacy. no BSC 5/2/24 for CR 3/12/24 ven. for for O.Dental ② No CR ③ Bone Scan	

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب



اسم المريض ثلاثي:

رقم الملف:

Doctor's Orders اوامر الطبيب

التشخيص: نوع التغذية:

التاريخ	الوقت	الأوامر	توقيع الطبيب
		Dentist Dear Dr, The patient has a tooth that needs to be extracted however she refused to do so and wishes to proceed with Zomen. Dr. Fatima Ali 15/1/2025	توقيع الطبيب

→ تم تسليمه على
15/1/25

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

اسم المريض ثلاثي:
 رقم الملف:



وزارة الصحة و السكان
امانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام " هرمل "
إدارة الجودة

Doctor's Orders الأوامر الطبيب

الفحص: نوع التغذية:

التاريخ	الوقت	الأوامر	توقيع الطبيب
22/6/2025		Done Scan → CR (No active lesion) R/Axillary 1x1x30 R/Ovar Area 1x1x90 R/Calcaneus 1x1x90 22/6/2025 [2x4.2ml of 2 units] أول 4 ديسا روغنيثا ٤ ورنيث زولاكس ٨/٨ عتاكس بنادول ٥ ٧x٥x١ سعيد C. C. A. IV سراج محمد	

Clinical pharmacy
Zameh Zaker
رئيس الصيدلانية
C. C. A. IV

جميع الأوامر الطبية لا بد أن تكون واضحة وموقعة من الطبيب



وزارة الصحة و السكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام " هرمل "
إدارة الجودة

أوامر الطبيب Doctor's Orders

اسم المريض ثلاثي:
عند المرض المعوي

رقم الملف :